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Anxiety and Related Disorders ×

Today's goal • 3/3 items

Module 1
Learn With PsycLearn Essentials

Module 2
Introduction to Psychological Disorders

Module 3
Opening to Anxiety Disorders

Module 4
Components of Anxiety

Module 5
Disorders that Involve Anxiety Part 1

Generalized Anxiety Disorders

- Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Generalized Anxiety Disorder
  Reading • 10 min
- Check Your Understanding: Generalized Anxiety Disorder
  Practice Assignment • Grade: 100%

Panic Disorder

- **Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Panic Disorder**
  Reading • 10 min
- Check Your Understanding: Panic Disorder
  Practice Assignment • 5 min

Specific Phobias

- Diagnostic Criteria and Prevalence for Specific Phobias
  Reading • 15 min
- Contributing Factors for Specific Phobias
  Reading • 10 min
- Classical Conditioning of Specific Phobias
  Video • 2 min
- Operant Conditioning and Negative Reinforcement of Specific Phobias
  Video • 3 min
- Cultural Issues for Specific Phobias
  Reading • 2 min
- Check Your Understanding: Specific Phobias
  Practice Assignment • 6 min

Key Takeaways

- Key Takeaways: Disorders that Involve Anxiety Part 1
  Reading • 10 min

Module 5 Quiz

- Module Quiz: Disorders that Involve Anxiety Part 1
  Graded Assignment • 10 min

Module 6
Disorders that Involve Anxiety Part 2

Module 7
Conclusion

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Module 9
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Module 10
PsycLearn Essentials APA Student Resources

Diagnostic Criteria, Prevalence, Contributing Factors, and Cultural Issues for Panic Disorder

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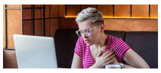

Panic Disorder Example

Carrie, a woman in her early 20s, was surprised one day when she felt her heart speeding out of control. The chest pains that accompanied her racing heart convinced her that she was having a heart attack. Terrified that she was going to die, she rushed to the emergency room, where doctors closely monitored her cardiac activity. Other than the fact that her heart was beating at an unusually fast rate, as if she had been exercising vigorously, Carrie's tests showed that her heart was completely normal. She wasn't having a heart attack or any other cardiac issue. After staying in the emergency room for a few hours to make sure the episode wouldn't recur, Carrie was sent home.

Despite the ER staff's reassurances, Carrie remained convinced that something was seriously wrong with her heart. When she followed up with her doctor and expressed her concern that the ER doctors missed something, she was referred to a heart specialist. The cardiologist prescribed a monitoring device to record Carrie's heart's activity. That test came back completely normal, as well. The cardiologist gave Carrie the good news that her heart was fine and suggested she follow up with a mental health care professional and get evaluated for panic attacks.

Carrie's story is quite common; many people experiencing panic attacks misperceive the episode as a heart attack and arrive in an emergency room believing they are on the brink of death. The physical symptoms of a panic attack—a fast heart rate, shortness of breath, dizziness—are understandably very frightening. In turn, the intense fear generated by the panic attack terrifies the person into believing that whatever is happening will lead to disastrous consequences—likely death.

Eventually Carrie followed up with a psychologist and was diagnosed with panic disorder. Carrie did indeed go on to have many panic attacks and, despite understanding that she had panic disorder, she was unable to shake the idea that there was something wrong with her heart. The fear resulting from the attacks led to hypersensitivity about any slight variations in her bodily sensations, and she interpreted them as evidence that she was about to die. Carrie lived in fear of having a panic attack.

Ultimately, Carrie was able to manage her panic disorder with a combination of psychotherapy and medication. Her panic attacks are much less frequent now, and she is able to talk herself out of believing that variations in her normal body rhythms are indicators that something is very wrong with her heart.

Diagnostic Criteria

Panic disorder is an anxiety disorder in which people experience **panic attacks**—terrifying episodes consisting of an extreme feeling of fear accompanied by a very high level of physiological arousal (e.g., a pounding heart, sweating, and light-headedness). As the name suggests, when panic attacks occur, people experience an overwhelming sense of fear; many panic attack sufferers believe they are dying. The fear created by panic attacks is so great that people who experience them often describe it as the most frightened they have ever been. As a result, sufferers live in fear of having another attack. Adding to the fear is the unpredictability of the attacks. Panic attacks can occur at any moment, without warning and often without any obvious cause. These terrifying episodes can spring up when you are on a date, when you are taking a final exam or, if you are an all-star basketball athlete playing in the third quarter of an important game.

Suffering from panic attacks is a key symptom of panic disorder, however in order to be diagnosed with panic disorder people must also experience negative consequences from having the attacks. Specifically, they need to either experience persistent anxiety about the possibility of having another panic attack. In some cases, if there is an awareness of what might cause a panic attack, some people might change their behavior to avoid these situations.

When taken to an extreme, these avoidance behaviors may meet the criteria for another psychological disorder, **agoraphobia**—a condition in which people actively avoid situations, such as being in a crowd or being outside the home alone, due to the fear of having a panic attack. In severe cases, agoraphobia can leave its sufferers homebound, unwilling to leave for fear of inducing a panic attack. Panic disorder and agoraphobia can each exist independently, but they are frequently comorbid.

Because panic attacks usually occur without warning and without obvious causes, people experiencing their first attack often don't realize what is happening. In fact, it's very common (and quite reasonable) for people to interpret the extreme physiological activation (heart pounding, shortness of breath, chest pains) as signs of a heart attack. Typically, it's only after a visit to the emergency room confirms that their cardiovascular system is healthy that physicians and patients start to consider the possibility that the reason for their physiological symptoms was a panic attack.

Prevalence

The global lifetime prevalence of panic disorder is approximately 1.7% (de Jonge et al., 2016). Prevalence is higher in the United States (4.7%) than in many other regions, including South America (2.2%), East Asia and Australia (2.0%), Western Europe (1.9%), the eastern mediterranean region (1.0%), Eastern Europe (0.9%), and Africa (0.2%). High income countries like the United States, Japan, and Belgium have a higher prevalence (2.2%) than countries with lower GDP such as Colombia, Iraq, and China (~0.8%). In the United States, white Americans have the highest prevalence rate with Asian, Latina/o, and African Americans all showing lower rates (APA, 2013).

Though estimates vary study to study, most estimate that women are 1.5 to 2.5 times more likely to receive a diagnosis of panic disorder as are men (Eaton et al., 1994; Kessler et al., 2005). There are multiple potential explanations for this difference including the facts that women are more sensitive to oxygen deprivation than men and tend to have more separation anxiety as well (Leskin & Sheikh, 2004). Although there is not a firm scientific consensus on the reason for this gender difference, the magnitude of the difference is largely consistent across cultures, suggesting that it is not a function of gender norms specific to one or a small number of cultures (Gater et al., 1998).

Contributing Factors of Panic Disorder

Biological Factors

The evidence for the role of genetics in panic disorder is relatively straightforward. Like other anxiety disorders, panic disorder tends to run in families. The likelihood of someone suffering from panic disorder is 3 to 5 times higher in families where at least one first-degree relative (i.e., a parent, child, or sibling) has the disorder (Weissman, 1993). Consistent with this idea, several studies have shown that panic disorder has higher concordance rates (if one twin has a characteristic, the likelihood that the other twin also has the characteristic) in identical twins than fraternal twins (e.g., Perna et al., 1997). Identical twins share 100% of their genes, while fraternal twins share only about 50%. The higher concordance rates of panic disorder for identical twins indicates that the reason panic disorder runs in families must have a genetic component—as opposed to arising solely from non-biological sources like a family living in a stressful environment or children learning to panic by watching their parents.

As is true with GAD, researchers have identified numerous structures in the brain involved in the generation of panic attacks. These structures include the amygdala, an area associated with the generation and experience of fear; the locus coeruleus, an area that can intensify fear effects; and the prefrontal cortex, an area involved in the regulation of emotion; along with nearly a dozen others (Butler et al., 1990; Gorman et al., 2000; Lai, 2019). Collectively, these areas are referred to as the **fear circuit**. Because of the important roles that the neurotransmitters noradrenaline (Charney et al., 1987) and serotonin (Maron & Shlik, 2006) play in the fear circuit, many psychopharmacological treatments for anxiety in general and panic disorder in particular attempt to regulate those systems. Recall the image of the fear circuit that appeared earlier.

Environmental Factors

Panic Disorder arises also from cognitive factors—how people think about themselves and the world. How we experience our bodily reactions to minor stressors can also affect our likelihood of having a panic attack. Individual differences in **anxiety sensitivity**—the awareness of, and concern about, sensations of anxiety arising from our bodies—play an important role in the creation of panic. As the name implies, people who are higher on anxiety sensitivity are more likely to interpret normal physiological variations, such as slight heart rate increases due to walking up a flight of stairs, as indicating potentially catastrophic health problems. In turn, this oversensitivity can set off a chain reaction of worry—a person gets concerned they are having a heart attack, that anxiety increases their heart rate further fueling their anxiety about their heart—ultimately leading to a panic attack (Asmundson & Norton, 1993; Cox, 1996).

Cultural Issues

As is true of GAD, the presentation of symptoms of panic disorder can be influenced by cultural issues, often through the personal explanations or attributions for the cause of the symptoms a person is experiencing (APA. 2022; ICD, 2019). For example, Cambodian individuals who have been traumatized by experiences as a refugee may attribute panic symptoms to dysregulation of khyâl, a wind-like substance that produces symptoms such as dizziness, tinnitus, and neck soreness (Hinton et al., 2010).

Other cultures, too, have explanations for panic symptoms specific to aspects of regional or national culture. Panic in Latin America may manifest itself in an *ataque de nervios* (an intense emotional experience, typically accompanied by yelling, crying, and trembling), but in Nigeria the same anxiety may present as a case of *ode ori*, in which the person reports feeling like something is crawling around inside their brain (Makanjuola, 1987). For a diagnosis of panic disorder, the response of panic must be excessive and unexpected. Decisions regarding the nature of an individual's panic must be informed by knowledge of cultural influences that provide context in which to understand the symptoms.

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